

COMPREHENSIVE 10-COLOR IMMUNOPHENOTYPING FOR B-LYMPHOBLASTIC LEUKEMIA

Patient Name:	Aarav Gowda	Date of Exam:	September 14, 2026
MRN / Patient ID:	KMIO-PED-2026-1209	Age / Gender:	8 Yrs / Male
Blood Group & Rh:	O+ (Rh Positive)	Referring Doctor:	Dr. Arun Kumar, MD (Pediatric Oncology)
Clinical Diagnosis:	Newly Diagnosed Acute Lymphoblastic Leukemia	Specimen / Source:	Bone Marrow Aspirate (Sodium Heparin)

Antigen Marker	% Positive Blasts	Intensity Level	Diagnostic Implication	Result
CD19 B-Lineage	98.6 %	Bright Uniform	Pan B-Cell Marker	Lineage Defining Positive
CD10 (CALLA)	96.4 %	Bright Positive	Common ALL Antigen	Common B-ALL Phenotype
Cytoplasmic CD79a	94.2 %	Positive	Early B-Precursor	Lineage Defining Positive
CD34 Progenitor	84.5 %	Moderate Positive	Stem / Progenitor	Immature Blasts
CD20 B-Cell	12.8 %	Dim Heterogeneous	Mature B-Cell	Pre-B Stage Maturation
Cytoplasmic IgM	< 1.0 %	Negative	Pre-B Differentiation	Consistent with Common B-ALL

CLINICAL INTERPRETATION & RECOMMENDATIONS:

- Gated lymphoblast population (82% of marrow events) exhibits diagnostic phenotype of Common B-ALL.
- Uniform bright co-expression of CD19, CD10, cytoplasmic CD79a, and CD34.
- Negative for myeloid (CD13, CD33, MPO) and T-cell (CD3, CD7) antigens.
- Recommendation: Initiate ICICLE / BFM induction protocol; send bone marrow for cytogenetics.

Dr. Manjunath Shenoy, MD (Hematology)
Medical Technologist / Pathologist

Dr. L. Appaji, MD (Professor & Head Pediatric Oncology)
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